

ENCOUNTER PROGRESS NOTE

PATIENT

Name: Evelyn Kowalski
DOB: 1941-11-05
MRN: MRN-2026-0419
Date of Service: 2026-04-28
Provider: Midwest Cardiac & Specialty Group
Visit Type: Office / Outpatient

CHIEF COMPLAINT

Two-day history of fever, altered mental status, and decreased oral intake. Brought in by family for evaluation.

HISTORY OF PRESENT ILLNESS

Mrs. Kowalski is an 84-year-old female who was in her usual state of health until 48 hours ago, when she developed subjective fevers, chills, and rigors. Her family reports she became progressively lethargic and confused, with poor oral intake. She has had two days of dysuria and urinary frequency, suggestive of a urinary source. No abdominal pain, no cough, no diarrhea.

PAST MEDICAL HISTORY

Hypertension, mild cognitive impairment at baseline (alert, oriented x3), osteoarthritis. No prior history of sepsis. Baseline creatinine 0.9 mg/dL per chart 2 months ago.

PHYSICAL EXAMINATION

Temp 102.1 degF, BP 88/52 (hypotensive -- baseline 130s/80s), HR 118, RR 22, SpO2 95% on RA. Patient is lethargic but arousable, oriented to person only (not place or time -- change from baseline). Dry mucous membranes, tenting skin turgor. Mild suprapubic tenderness. No flank tenderness. Lungs clear. Heart tachycardic but regular. No focal neurologic deficit.

LABS / IMAGING

WBC 18.5 with 92% neutrophils and bandemia. Serum lactate 3.2 mmol/L (elevated, normal <2.0). Procalcitonin 4.8 (elevated). Creatinine 2.4 mg/dL -- a sharp rise from baseline 0.9 within 48 hours, consistent with acute kidney injury (KDIGO Stage 2). BUN 52. Urinalysis: large leukocyte esterase, positive nitrites, >100 WBC per HPF -- confirmed urinary source of infection. Blood cultures x 2 drawn prior to antibiotics. CXR clear.

ASSESSMENT

- Sepsis, unspecified organism (A41.9). The patient meets SIRS criteria (fever, tachycardia, tachypnea, leukocytosis with bandemia) with a documented urinary source of infection and elevated lactate of 3.2. Hypotension and altered mental status are consistent with sepsis-induced organ dysfunction.
- Acute kidney injury (N17.9). Documented creatinine rise from baseline 0.9 to 2.4 mg/dL within 48 hours -- meets KDIGO criteria (creatinine increase ≥ 0.3 mg/dL within 48h and $\geq 1.5\times$ baseline within 7 days). Likely pre-renal in the setting of sepsis and volume depletion.
- Dehydration (E86.0).

PLAN

IV fluid bolus 2 L normal saline now. Broad-spectrum antibiotics: ceftriaxone 2 g IV (covers urinary pathogens). Central venous catheter placed via right internal jugular under ultrasound guidance for fluid resuscitation and vasopressor access if needed. Admit to step-down unit. Re-image lactate in 2 hours; reassess fluid status. Nephrology consult for AKI.

PROVIDER SIGNATURE

Electronically signed by Dr. Elena Vasquez, MD on 2026-04-28.